

MRI Space Mission: A Stepwise Paediatric MRI Training Protocol for Young Children



Hsin-Yu Lin¹, Chiao-Yi Wu^{1,2}, JiaLi Teo³, Phoebe Si Qi Chia^{3,4}, Michelle Li-Mei Yap², Marilyn Cai Ling Yeo², Suzy. J. Styles³, Shen-Hsing Annabel Chen^{1,3,4}

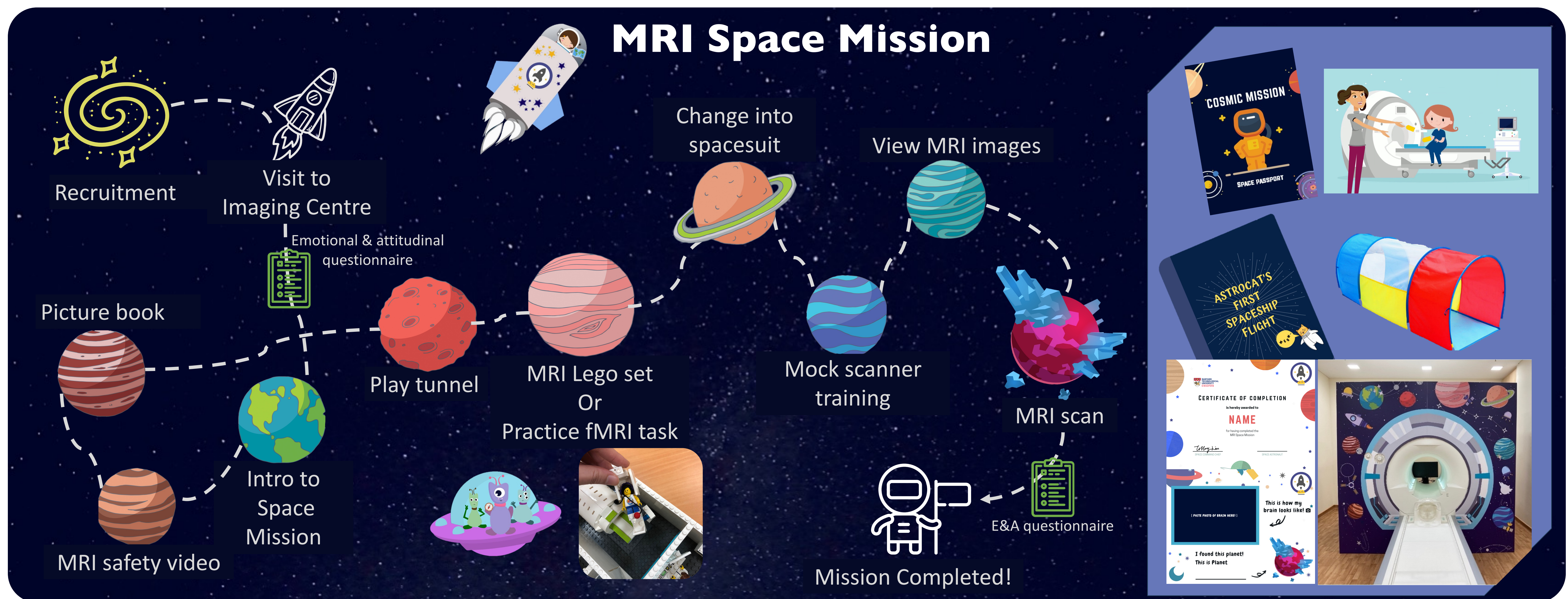
¹Centre for Research and Development in Learning, Nanyang Technological University, Singapore, ²Centre for Research in Child Development, Office of Education Research, National Institute of Education, Nanyang Technological University, Singapore, ³Psychology, School of Social Sciences, Nanyang Technological University, Singapore, ⁴Centre for Sleep and Cognition, Multimodal Neuroimaging in Neuropsychiatric Disorders Laboratory, Yong Loo Lin School of Medicine, National University of Singapore, Singapore, Lee Kong Chian School of Medicine, Nanyang Technological University, Singapore

Introduction

- Magnetic Resonance Imaging (MRI) can be particularly challenging for young children, as it requires them to remain still for an extended period while lying inside a loud and enclosed scanner.
- Many children experience anxiety or fear when faced with the large, unfamiliar machine and the noises it produces [1].
- Movement during scanning often leads to motion artefacts, reducing image quality and sometimes necessitating scan repetition or sedation [2].
- Previous studies have shown that playful preparation can reduce anxiety and sedation rates in paediatric MRI [3,4].
- To address these challenges, we developed the **MRI Space Mission**, a child-centred, stepwise **paediatric MRI training protocol**.

Aim

- To improve scan success rates by familiarising children with the MRI environment in a playful way and reducing procedure-related anxiety.



Participants & MRI data

- 48 (5-7 y) and 40 (7-9 y) kindergarten and primary school children
- TIw (TR/TE = 2000/2.26); Functional (TR/TE = 1500/30); Diffusion (TR/TE = 5000/91)
- 3T Siemens Prisma scanner, 32-channel head coil
- fMRI (SPM12 [6]): Motion detected by ARTrepair [7] (threshold = 0.6 mm; run excluded if >25% bad volumes). A dataset was considered successful if at least one language has >2/4 valid runs.
- DSI (DSI Studio [5]): Data were considered unsuccessful if neighbouring DWI correlation <0.7 or >15 bad slices.

Results

Sequence	Storyline video	Study 1 (5-7y, n=48)	Study 2 (7-9y, n=40)	Duration (mm:ss)
MRI scan completion rate		97.92%	100%	-
	Sound Check video	-	-	00:26
Localizer	Launching off video	-	-	00:14
T1	Cartoon	64.58%	82.5%	04:40
Field maps	Task/Water galaxy intro	-	-	01:24
Resting-state	Inscape video	62.5%	N/A	07:10
fMRI	Language games (8 runs)	N/A	80%	21:20
T2	Arrival at mystery planet	70.83%	70%	02:14
DSI AP, PA	Continue cartoon	43.75%	45%	12:50
Total duration		27:52	42:42	Max 1 hr

Emotional & Attitudinal Ratings (5 points scale)		Study 1	Study 2
Emotional statements (child)		No significant pre-post changes	
Emotional statements (parent)*		after	secure↑; nervous↓ worried↓ worried↓; unsure↓ unsure↓
I know about the MRI spaceship (child)*		before	2(1.45) 2.13(1.4)
		after	4.21(1.1) 4.51(0.72)
I'm familiar with what the MRI machine does (parent)*		before	3.54(1.04) 3.17(0.89)
		after	4.52(0.69) 4.12(0.6)
I feel uncertain about my child going through an MRI scan (parent)*		before	2.38(1.11) 2.23(0.88)
		after	0.95(1.81) 1.6(0.61)

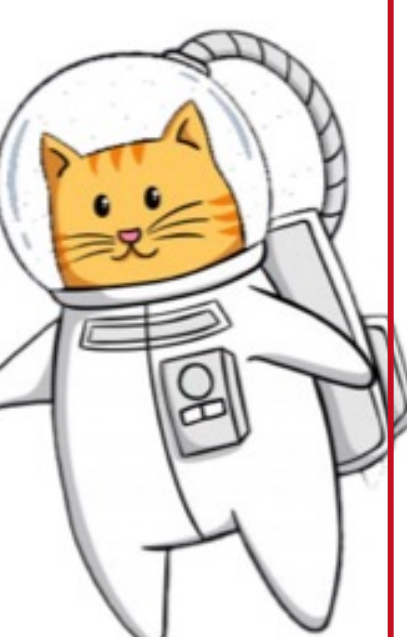
Paired t-tests were done for all items (before vs. after). *p < 0.0036, Bonferroni corrected.

Conclusions

- The 98.86% attendance rate and the high activity ratings indicated that our MRI training protocol effectively increased both children's and parents' willingness to participate in an MRI study.
- Overall, our completion rates were higher than those reported in previous studies, and our sequence-level success rates were comparable to or better than prior paediatric MRI work that implemented child-friendly preparation [2,3,8,9].
- Our space-themed MRI training protocol is feasible and effective.
- However, the results suggest that there is still room to further reduce head motion and improve children's ability to remain still with additional measures.



Scan to watch



References

- [1] O'Shaughnessy, E. S. (2008). J Child Neurol. 23(7):791-801. [5] Yeh, F. C. (2025). Nat Methods. 22(8):1617-1619. (RRID:SCR_005990). www.nitrc.org/projects/art_repair/ [2] Thieba, C. (2018). Front Pediatr. 6:146. [6] Wellcome Department of Imaging Neuroscience. [8] de Bie, H. M. A. (2010). Eur J [3] Rajagopal, A. (2014). AJNR Am J Neuroradiol. 35(12):2319-2325. (www.fil.ion.ucl.ac.uk/spm) Pediatr. 169:1079-1085. [9] Theys C. PLOS ONE. 2014;9(4):e94019.

Contact information: Tiffany Hsin-Yu Lin, linhy@ntu.edu.sg

